



Patient Detail:

Areeba Dilshad .

Age/Sex :

21 (Y) / F

Registration Location:

Near Jinnah Hospital, Lahore

Registration Date:

16-May-2026 13:50

Reference:

Diabetes Care Program

Consultant:

.Dr Usman Asad Khan

Patient Number:

38001-25-16626515

Case Number:

45065-16-05



NIC #: 36601-1619743-3,,,

Department of Radiology

Reporting Date/Time: 16-May-2026 16:15

Abdominal Pelvis Ultrasound

Liver is not enlarged measures 160 mm in the mid clavicular line (Normal upto 160 mm), and shows normal echotexture with smooth contours. No focal lesion. Intra hepatic biliary channels are not dilated.

Portal vein measures 8 mm and is patent.

CBD measures 3 mm.

Gall Bladder wall is of normal thickness. No stone or sludge. No mass seen.

Pancreas is normal in size, echotexture and outline in its visualized portion.

Spleen is not enlarged measures 90 mm (normal <120 mm). No mass seen.

Rt. Kidney	109 x 35 mm	Cortical thickness measures 10 mm
Lt. Kidney	113 x 40 mm	Cortical thickness measures 12 mm

Renal outlines are smooth. Normal cortico-medullary demarcation. No calculus, mass or hydronephrosis.

Urinary Bladder wall is of normal thickness. No stone or mass seen.

No abdominal mass, ascites or enlarged lymph nodes seen.

No distended bowel loops. Normal peristalsis.

No basal pleural effusion seen.

Pre-void urine measures 380 ml.

Post-void urine measures 25 ml ----- after visiting the washroom twice.

Anteverted uterus	82 x 33 x 43 mm	with normal contour and endometrium measures 10 mm, thick three lines.
Rt. Ovary	34 x 16 x 35 mm	Volume: 10 ml

A well defined unilocular cyst with internal echoes and without any solid component is seen in the left adnexal region measuring 41 x 30 x 40 mm (Volume: 26 ml). No blood flow on doppler.

Another cyst of similar nature is seen in the cul de sac measuring 33 x 27 x 35 mm (Volume: 16 ml). No blood flow on doppler.

Left ovary is not visualized separately from these cysts. Right ovary is normal.

No uterine mass seen.

No free fluid in culde sac.

IMPRESSION	No evidence of hepatobiliary, pancreatic or renal pathology. Complex cysts left adnexal region and cul de sac most likely hemorrhagic cysts. No bladder outflow obstruction (PMRV 25 ml).
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This interpretation is based on available information. Correlation with clinical assessment, previous imaging and laboratory investigations is suggested. The study can be reviewed in case of any clinical query.

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